

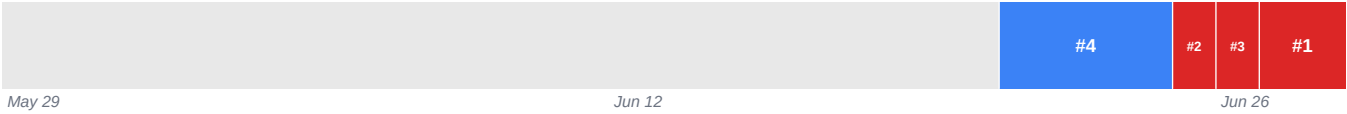
CLINICAL INTELLIGENCE TREND REPORT

RED: Provider Review Recommended

Patient ID: S (Chair) Period: May 29 to June 28, 2024 Report Date: June 29, 2024 Coverage: 584/736h (76.8%)
Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 31 days from May 29 to Jun 28

■ HR ■ Breathing



Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
15	39h	High Heart Rate	<1	79%

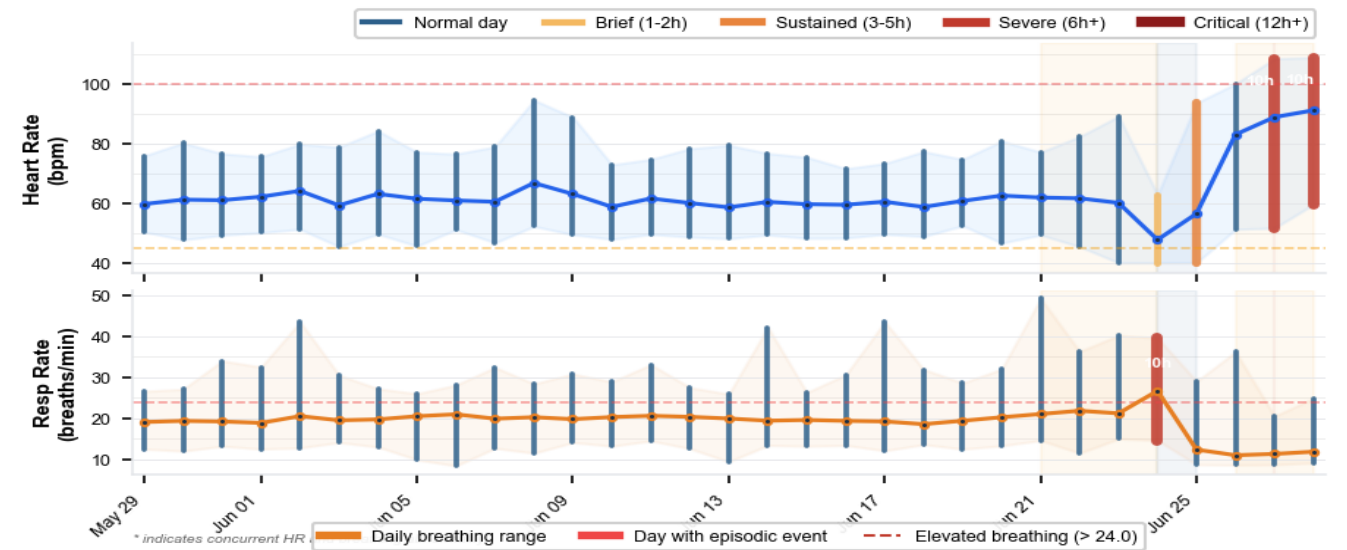
Major Findings: Sustained high HR (avg 89, peak 109)

#	Episode	Min/Max	Total Hours	Longest Continuous	Episodes/d ay	Average	Date
1	High Heart Rate	109 bpm	7h	7h	1	89 bpm	Jun 27
2	Low Heart Rate	40 bpm	4h	3h	1	52 bpm	Jun 24 to Jun 25
3	Elevated Heart Rate	106 bpm	15h	3h	3	88 bpm	Jun 26 to Jun 28
4	Elevated Breathing	44 brpm	13h	10h	1	23 brpm	Jun 21 to Jun 24

The P5 to P95 heart rate spread was 40 bpm (53 to 93), indicating significant cardiac variability.

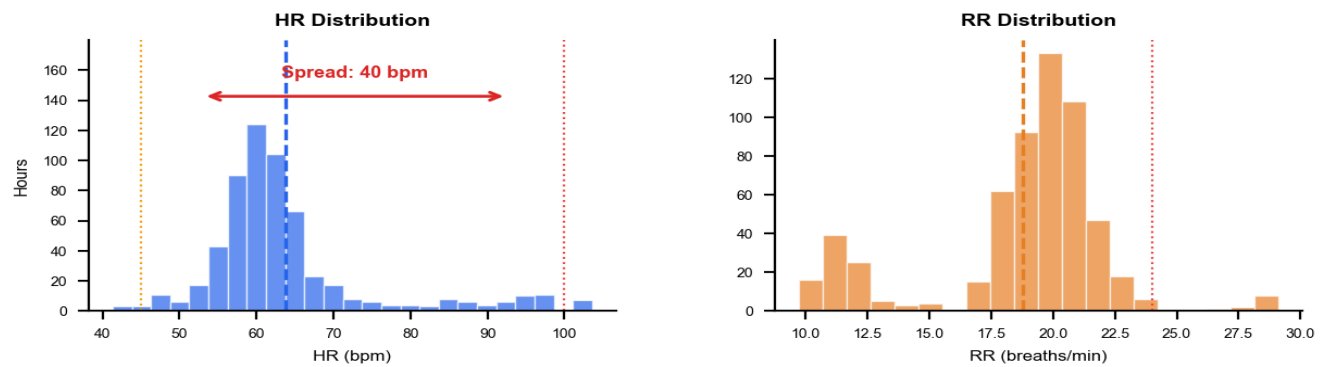
Clinical Pattern Observations:

- 1. **Sustained finding:** 10h continuous Elevated Breathing on Jun 24.
- 2. **High variability:** HR spread 40 bpm (52 to 92).
- 3. **Breathing variability:** RR spread 11 brpm (11 to 22).

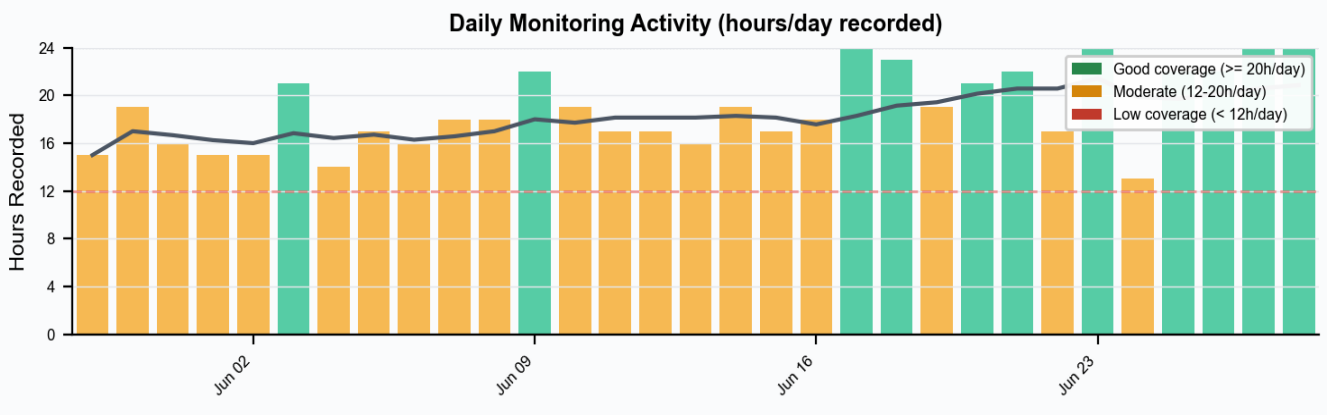


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

■ Very Low HR < 40 bpm ■ Low HR < 45 bpm ■ Elevated HR > 95 bpm ■ High HR > 100 bpm
■ Very High HR > 110 bpm ■ Elevated Breathing > 24 brpm ■ High Breathing > 30 brpm ■ Very High Breathing > 40 brpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.